

Medications

omeprazole Oral
metformin Oral

Medical History

Type 2 diabetes mellitus
Gastroesophageal reflux disease

Musculoskeletal History

Chronic low back pain

Musculoskeletal Family History

None

Musculoskeletal Surgery

None

Surgical History

None

Social History

Smoking status - Former smoker

Chief Complaints:

1. Low back pain radiating into the left leg, 6 weeks.

HPI: This is a 50 year old male who:

Patient reports gradual onset low back pain six weeks ago after lifting a heavy box at work, now radiating down the posterior left thigh into the calf. Pain worsens with prolonged sitting and forward flexion, improves lying supine. Denies bowel or bladder dysfunction, saddle anesthesia, or bilateral leg weakness. Has tried ibuprofen with minimal relief.

Vitals:

Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI BSA
05/14/25	T. Nguyen, MA	138/88	78	16	97	98.4	70.0 in	214.6 lbs	30.8 2.15

Exam:

Comprehensive, Lower Extremity Neurovascular

Appearance: well nourished

Orientation: Alert and oriented to person, place, time.

Mood: mood and affect well-adjusted, pleasant and cooperative,

Right LE Pulses: normal dorsalis pedis pulse and good capillary refill

Left LE Pulses: normal dorsalis pedis pulse and good capillary refill

RLE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

LLE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

Lumbar Spine

Lumbar Spine Active ROM:

Flexion: 45 degrees.

Extension: 15 degrees.

Right Lateral Bending: 25 degrees.

Left Lateral Bending: 25 degrees.

Lumbar Spine Passive ROM:

Flexion: 60 degrees.

Extension: 25 degrees.

Right Lateral Bending: 25 degrees.

Left Lateral Bending: 25 degrees.

Skin:

Lumbar Spine: skin intact, no rashes or lesions.

Inspection:

Lumbar Spine: paraspinal tenderness at L5-S1, no midline step-off

Strength:

Lumbar Spine Flexion: Strength: 5/5, normal muscle tone.

Lumbar Spine Extension: Strength: 5/5, normal muscle tone.

Stability:

Lumbar Spine: No instability

Special:

Lumbar Spine: Straight leg raise: positive on the left at 40 degrees

Tests

X-Ray Interpretation Lumbar Spine

Diagnosis: Low Back Pain, Left - M51.16

X-Ray Data:

Date: 05/14/2025

The following film(s) were done in our facility: lumbar spine AP view and lateral view

Two views of the lumbar spine. Mild multilevel degenerative disc space narrowing. No fracture, spondylolisthesis, or acute osseous abnormality.

Impression: Mild lumbar spondylosis; no acute osseous findings.

Impression/Plan:

1. Low Back Pain, Left

Lumbar disc herniation with left-sided radiculopathy (M51.16)

Associated diagnosis: Lumbar radiculopathy (M54.16)

Plan: Prescription.

methocarbamol 750 mg tablet PO

Sig: Take 1 po tid x 10 days

Quantity: 30 Tablet Refills: 0

Follow up in 6 weeks

Note:

Findings consistent with left L5-S1 radiculopathy. Ordered lumbar spine x-rays today and will proceed to MRI given persistent radicular findings. Started methocarbamol for muscle spasm and discussed activity modification. Will reassess after imaging is complete.

Staff:

Dorian Vance, MD (Primary Provider) (Bill Under)

Electronically Signed By: Dorian Vance, MD, 05/14/2025 10:15 AM CDT

Medications

omeprazole Oral
metformin Oral
methocarbamol Oral

Medical History

Type 2 diabetes mellitus
Gastroesophageal reflux disease

Musculoskeletal History

Chronic low back pain

Musculoskeletal Family History

None

Musculoskeletal Surgery

None

Surgical History

None

Social History

Smoking status - Former smoker

Chief Complaints:

1. Follow-up low back pain with left leg radiculopathy, status post imaging.

HPI: This is a 50 year old male who:

Patient returns after lumbar MRI obtained 5/20/2025 confirming a left paracentral disc herniation at L5-S1. Radicular pain into the left leg persists despite methocarbamol and activity modification, now rated 7/10 and affecting sleep. Denies new weakness, bowel or bladder changes.

Vitals:

Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI BSA
06/25/25	T. Nguyen, MA	134/86	74	16	98	98.1	70.0 in	212.0 lbs	30.4 2.13

Exam:

Comprehensive, Lower Extremity Neurovascular
Appearance: well nourished
Orientation: Alert and oriented to person, place, time.
Mood: mood and affect well-adjusted, pleasant and cooperative,
Right LE Pulses: normal dorsalis pedis pulse and good capillary refill
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Inspection:

Lumbar Spine: paraspinal tenderness at L5-S1, no midline step-off

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Lumbar Spine Extension: Strength: 5/5, normal muscle tone.

Stability:

Lumbar Spine: No instability

Special:

Lumbar Spine: Straight leg raise: positive on the left at 40 degrees

Tests

MRI Interpretation Lumbar Spine
Diagnosis: Low Back Pain, Left - M51.16
MRI Data:
Date: 05/20/2025
MRI of the lumbar spine without contrast. Left paracentral disc herniation at L5-S1 with left lateral recess narrowing and contact with the descending left S1 nerve root. No central canal stenosis.
Impression: Left L5-S1 disc herniation with left S1 nerve root impingement.

Impression/Plan:

1. Low Back Pain, Left
Lumbar disc herniation with left-sided radiculopathy (M51.16)
Associated diagnosis: Lumbar radiculopathy (M54.16)

Plan: Prescription.
gabapentin 300 mg capsule PO
Sig: Take 1 po tid
Quantity: 90 Capsule Refills: 1

Follow up in 3 weeks

Note:

MRI confirms left L5-S1 disc herniation correlating with exam. Given persistent radiculopathy despite six weeks of conservative care, discussed surgical options. Patient elects to proceed with left L5-S1 microdiscectomy. Starting gabapentin for neuropathic pain while surgery is scheduled.

Staff:

Dorian Vance, MD (Primary Provider) (Bill Under)
Electronically Signed By: Dorian Vance, MD, 06/25/2025 09:00 AM CDT

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metformin Oral
methocarbamol Oral
gabapentin Oral

Medical History

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Gastroesophageal reflux disease

Musculoskeletal History

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Musculoskeletal Family History

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Surgical History

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Social History

Smoking status - Former smoker

Chief Complaints:

1. Post-operative follow-up, left L5-S1 microdiscectomy.

HPI: This is a 50 year old male who:

Patient is two weeks status post left L5-S1 microdiscectomy performed 07/02/2025. Reports significant improvement in radicular leg pain, now rated 2/10, with residual mild incisional soreness. Ambulating independently and tolerating gabapentin taper. Denies fever, wound drainage, or new neurologic symptoms.

Vitals:

Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI BSA
07/16/25	T. Nguyen, MA	128/80	70	16	98	98.0	70.0 in	209.5 lbs	30.1 2.11

Exam:

Comprehensive, Lower Extremity Neurovascular
Appearance: well nourished
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Stability:
Lumbar Spine: No instability

Special:
Lumbar Spine: Straight leg raise: positive on the left at 40 degrees

Operative Note

Procedure: Left L5-S1 microdiscectomy

Date: 07/02/2025

Surgeon: Dorian Vance, MD

Left L5-S1 microdiscectomy performed 07/02/2025 under general anesthesia. Estimated blood loss 25 mL. Fragment of extruded disc material removed from the left lateral recess. No dural tear. Patient tolerated the procedure well.

Impression/Plan:

1. Low Back Pain, Left

Lumbar disc herniation with left-sided radiculopathy, status post microdiscectomy (M51.16)

Associated diagnosis: Lumbar radiculopathy, improving (M54.16)

Plan: Prescription.

tramadol 50 mg tablet PO

Sig: Take 1 po q6h PRN pain

Quantity: 20 Tablet Refills: 0

Plan: Post-operative Care.

Follow up in 2 weeks

Note:

Excellent early post-operative recovery with resolution of preoperative radicular symptoms. Continue tramadol as needed for residual soreness and begin a gentle home exercise program. Cleared to increase activity gradually.

Staff:

Dorian Vance, MD (Primary Provider) (Bill Under)

Electronically Signed By: Dorian Vance, MD, 07/16/2025 01:30 PM CDT